

Lean Canvas

Designed for:

MSS Project

Designed by:

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V1.0

Problem - Limited access to maternity healthcare services in regional area. - Long travel time for expectant mothers. - Inequitable access to healthcare service particularly for remote aboriginal and culturally diverse communities.	Solution - Remote health consultation and patient monitoring. - Integrated appointment scheduling for patients and healthcare providers. - Integrated billing system. - Data security, access management and privacy-compliant by design. - Patient information (i.e. case history, report, prescription) management & exchange	Unique Value Proposition MSS enables expectant mothers in remote areas of WA to access critical, specialised and general maternity services without the need for physical travel, while also promoting greater access to healthcare for remote, Indigenous, and culturally diverse communities.	Unfair Advantage - Early-mover advantage in regional maternity healthcare, with a clear pathway to expand into other Australian markets. - Strong social and cultural inclusivity, specifically designed to address the needs of Aboriginal and CALD communities. - Already established commercial relation between MSI and WA Country Health Service - Integrated health information management and exchange system connecting key maternity services through a unified platform	Customer Segments - Expectant mothers in Perth, Rockingham, Mandurah and regional areas in WA - The Aboriginal and culturally and linguistically diverse (CALD) communities - Allied healthcare staff i.e. clinicians, midwives, obstetricians, radiologists, pathologists, pharmacists, and telehealth providers. - Hospital administrative staff - WA Country Health Service - Maternity clinics and hospitals in remote areas
Existing Alternatives - In person visits to GPs, clinics and hospitals. - Generic telehealth platforms lacking maternity specific services - Phone based health advice lines with limited documentation - Manual/paper-based or siloed patient information management systems i.e. standalone spreadsheets, local clinic databases, printed lab reports etc - Separate systems for pathology, imaging, pharmacy and telehealth	Key Metrics - Reduction in patient travel time - Number patient and allied healthcare staff onboard in the system - The appointment cancellation and no-show rate - System uptime and reliability - Telehealth consultation volume and completion rate - Security or privacy incidents - Revenue vs ROM benefit target - Percentage of project milestones delivered on time	Assumptions - Patients and healthcare staff have adequate internet connectivity and access to suitable devices - Patients and healthcare staff have sufficient digital literacy - Third-party providers can meet the required integration requirements and project deadlines - Pilot project can be delivered within the estimated timeframe and budget - Licensing, implementation and support cost remains within the ROM estimates - MSS meets required security, privacy and healthcare compliance standards	Channels - Community outreach through Aboriginal medical services, maternity women's network and local community workers - Integration with existing clinics and hospitals - Referral through WA Country Health service, local clinics and hospitals - Community awareness campaigns before go-live	Early Adopters - Expectant mothers in regional areas. - Maternity women's network members who are already involved in the steering committee - Allied healthcare staff who are already familiar with digital tools - Allied healthcare staff involved in the pilot project - Hospitals and clinics with existing digital healthcare infrastructure - WA Country Health Service sites

- *MSI can successfully complete the post-pilot handover to its IT support team through the P2S process*

Cost Structure

- *Software Licensing (38% of the budget)*
- *Implementation service (25% of the budget)*
- *User training & support (15% of the budget)*
- *Data migration & cleansing (10% of the budget)*
- *Loan servicing (5% of the budget)*
- *Contingency cost (10% of the budget)*
- *Network and infrastructure installation*

Revenue Structure

- *Service agreements/funds from WA Country Health Service*
- *Licensing revenue if MSS extends to the other parts of Australian market*
- *Government grant supporting regional and Aboriginal health access*

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